

Hypertension Follow-Up Protocol

Post-Discharge Management of High Blood Pressure | City General Hospital

1. Overview

Hypertension (BP $\geq$ 140/90 mmHg) is the leading modifiable risk factor for stroke, MI, heart failure, and chronic kidney disease. The first 4 weeks after discharge are critical for establishing effective blood pressure control.

Blood pressure is highly variable and influenced by activity, stress, sleep, diet, and medications. Home monitoring identifies patterns requiring clinical attention — keep a written record and bring it to every appointment.

2. Blood Pressure Monitoring Instructions

- Use a validated upper-arm automatic monitor (not a wrist monitor).
- Sit quietly for 5 minutes; back supported, feet flat, arm cuff at heart level.
- Avoid caffeine, tobacco, and exercise for 30 minutes before measuring.
- Take two readings 1–2 minutes apart; record the lower value.
- Measure twice daily: before morning medications and before evening meal.

3. Blood Pressure Target Goals

Patient Group	Home BP Target	Clinic BP Target	Monitoring Frequency
General adult (< 80 years)	< 130/80	< 140/90	Twice daily $\times$ 4 weeks
Elderly ($\geq$ 80 years)	< 150/90	< 150/90	Twice daily $\times$ 4 weeks
Diabetes + hypertension	< 125/75	< 130/80	Twice daily ongoing
Post-hypertensive crisis	< 130/80	< 135/85	Three times daily $\times$ 1 week

Home BP is typically 5–10 mmHg lower than clinic BP. Never self-adjust medication based on single home readings.

4. Medication Adherence

Poor adherence is the most common cause of uncontrolled hypertension. Missing doses — even occasionally — significantly increases stroke risk.

- Take BP medication at the same time daily; link it to a fixed routine.
- Never stop because readings look normal — that means the medication is working.
- ACE inhibitors: dry persistent cough is common; ask your doctor about switching to an ARB.
- Calcium channel blockers: ankle swelling is expected — do not stop without advice.
- Beta-blockers: never stop abruptly — always taper under medical supervision.

5. Lifestyle Modifications

Lifestyle changes are as effective as a single BP tablet and complement medications.

- Exercise: 150 minutes of moderate aerobic activity per week lowers systolic BP by 5–8 mmHg.
- Weight: losing 5–10 kg reduces systolic BP by up to 20 mmHg in overweight patients.
- Smoking: cessation is essential; smoking raises BP acutely with every cigarette.
- Alcohol: limit to ≤ 1 –2 units/day; excess alcohol is a common cause of resistant hypertension.
- Stress: mindfulness, yoga, and diaphragmatic breathing have measurable BP-lowering effects.

6. DASH Diet Recommendations

The DASH (Dietary Approaches to Stop Hypertension) diet reduces systolic BP by 8–14 mmHg. Key principles:

- Sodium: limit to < 1.5 g/day; avoid adding salt and check food labels.
- Potassium: eat bananas, oranges, potatoes, spinach daily — counteracts sodium's effect.
- Magnesium and calcium: found in leafy greens, nuts, seeds, and low-fat dairy.
- Reduce red meat; replace with oily fish, poultry, beans, and lentils.

7. Warning Signs — Hypertensive Emergency

- Call 112 immediately if BP > 180/120 mmHg with any of the following symptoms. These may indicate a hypertensive emergency.
- Sudden severe headache — 'worst headache of my life'; possible intracranial hypertension.

■ Visual disturbances: blurred vision, double vision, sudden loss of vision.

■ Facial drooping, arm weakness, or slurred speech — stroke symptoms.

■ Severe chest pain with or without shortness of breath — possible aortic dissection.

■ Nausea/vomiting with BP > 180/120 mmHg, or nosebleed that will not stop.

8. Hypertension Follow-Up Schedule

Time Point	Review Type	Key Tests	BP Target
Day 1–3	Nurse phone check	BP diary review	< 150/95
Day 7	GP visit	BP, medication review	< 145/90
Week 4	GP / specialist review	BP, renal function, K+	< 135/85
3 Months	Hypertension clinic	BP, ECG, bloods, urine protein	< 130/80
Annually	GP annual review	Full metabolic panel, CVD risk	< 130/80

9. Home Monitoring Log Instructions

- Morning: 2 readings before medications; record both with the time.
- Evening: 2 readings before dinner; note any relevant symptoms or activities.
- Weekly average: sum all readings ÷ number taken; target < 130/80 for most patients.

BP Nurse Helpline Mon–Fri 09:00–17:00. For BP emergencies or symptoms in Section 7, call 112. Synthetic document — for demonstration purposes only.